

**CALIMESA FIRE DEPARTMENT
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- d. Audit controlled substances
- 2. Copies of each DEA Form 222, including voided forms; purchase records; a log(s) of all controlled substances ordered, received, stored, damaged, placed in service, administer, wasted, restocked, returned to master supply, reverse distribution, and/or transferred or exchanged between agencies and/or services; and a patient care record/electric patient record (PCR/ePCR) or other appropriate report corresponding to each administration, waste, damage, or expiration.
 - a. CMS EMS-I — City of Calimesa Fire Department daily controlled substances inventory log. (Appendix B)
 - b. CMS EMS-2 — City of Calimesa Fire Department controlled substances restock request form. (Appendix C)
 - c. CMS EMS-3 — City of Calimesa Fire Department monthly controlled substances inventory log. (Appendix D)
- 3. Controlled substances PAR levels for all apparatuses, restock and specialties, set forth by REMSA policy 3303 and adopted by City of Calimesa Fire Department, will be always met.
- 4. These records will be:
 - a. Maintained at and/or electronically accessible from the master supply facility
 - b. Available for inspection within twenty-four (24) hours
 - c. Retain for a period of no less than three (3) years
 - d. Documents must be completed daily and available for inspection and review by the EMS Coordinator, Medical Director and local, state, or federal regulatory representatives to ensure compliance.

Vehicle Storage and Security

Make every reasonable attempt to follow the manufacturer's guidelines regarding vehicle storage of each controlled substance while in service:

- 1. Avoid exposure to extreme temperatures
- 2. Protect from light as required
- 3. Controlled substances will remain in original manufacturer's containers, Food and Drug Administration (FDA) compliant labels remaining intact and unaltered, until the time of administration.

Vehicle storage security measures will include:

- 1. To prevent the unauthorized access to our controlled substances during an incident, the on duty firefighter/paramedic must either have physical possession of the controlled substances container or the container is secured in the assigned EMS compartment on the engine with a minimum of two forms of locks.

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2. All REMSA personnel authorized to handle controlled substances shall follow this procedure.
 - a. All City of Calimesa Fire Department sworn safety personnel are authorized to witness narcotic accountability counts/and or controlled substance wasting.
 - b. All controlled substances are to be secured by the on-duty firefighter/paramedic in the EMS assigned engine compartment with a minimum of two forms of locks.
 - i. Option 1: controlled substances container with a padlock and a numerical red tag placed in the drug box and the EMS assigned engine compartment door locked.
 - ii. Option 2: on-duty firefighter/paramedic assigned to the engine will stay at the engine with the padlocked and numerically red tagged controlled substances container placed in the drug box.
 - c. The assigned on-duty shift paramedic shall maintain possession of the assigned EMS engine compartment throughout the course of the shift.
 - i. A backup key to the EMS assigned engine compartment will be secured in the "Knox" box next to the fire captain's seat.
 - d. At completion of each shift (every 48 hours) or when an on-duty firefighter/paramedic crew change occurs a controlled substance exchange is performed.
 - i. With both incoming and off going firefighter/paramedic's present both parties will verify through visualization of the clear controlled substance container the medication, quantity of medication, integrity of the medication vial or ampule, and the expiration dates of the controlled substance. Both parties will then inspect the controlled substance container for alterations and inconsistencies, padlock continuity and that the numerical red tag matches CMS EMS-I.
 - ii. If there is a discrepancy on any level regarding the controlled substance(s) par levels, medication(s), quantity of medication, integrity of the medication vial or ampule, the expiration dates or information on CMS EMS-I and/or CMS EMS-2 both incoming and off going firefighter/paramedics need to reconcile the discrepancy. If the discrepancy cannot be reconciled, immediately notify the on-duty and off going captains, on-duty and off going Battalion Chiefs and the EMS Coordinator. The EMS Coordinator will complete and submit a DEA Form 106, if necessary, and advise the Medical Director. (Report of Theft or Loss of Controlled Substance(s)).

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- iii. Both off-going and incoming will complete a narrative of the events.
- e. If the incoming firefighter/paramedic is dispatched to an incident before the controlled substance exchange can be completed, then the accountability will be completed as soon as possible and the off going firefighter/paramedic is released. Upon returning to the fire station and the off going firefighter/paramedic has left then the Duty Chief or EMS Coordinator may assist with the controlled substance exchange and documentation.
- f. In the event the on-duty assigned firefighter/paramedic works two consecutive shifts (96 hours) on the same apparatus then the accountability must be done with appropriate forms (CMS EMS-I) filled out after 48 hours.
- g. EMS Coordinator or Medical Director will conduct a monthly inventory reconciliation.
- h. Any inventory or reconciliation discrepancies shall immediately be brought to the attention of the Fire Chief, Medical Director, and EMS Coordinator. Form DEA 106, Report of theft or loss of a Controlled Substance, shall be completed and submitted (21CFR 1301.76).

Vehicle Documentation

1. A controlled substance logbook will be on each apparatus and will contain the following items.
 - a. CMS EMS 1 — City of Calimesa Fire Department Daily Controlled Substance
 - i. CMS EMS 2- City of Calimesa Fire Department Controlled Substance Restock Request
 - ii. REMSA 3302 — Controlled Substances
 - iii. RE-MSA 4105 -ALS Drug Index iv. City of Calimesa Fire Department Policy 505.01
 - b. Documents must be completed daily and available for inspection and review by the EMS Coordinator, Medical Director and local, state, or federal regulatory representatives to ensure compliance.

Accountability and Standard Controlled Substance Inventory

The standard par levels of a controlled substance(s) are established by REMSA protocol 3303.

1. The following is the City of Calimesa Fire Department's par levels for all apparatus:
 - a. Fentanyl (Sublimaze) — 200 mcg (2x 100 mcg/2 mL ampules or vials)

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- b. Ketamine (Ketalar) — 500 mg (1x 500 mcg/10 mL ampule or vial) or (1x 500 mcg/5 mL ampules or vials)
 - c. Midazolam (Versed) — 20 mg (4x 5mg/1 mL ampules or vials)
 - d. Morphine Sulfate (alternative) N/A
- 2. The following is the City of Calimesa Fire Department's par levels for the Master Supply Storage:
 - a. Fentanyl (Sublimaze) — 1000 mcg (10x 100 mcg/ 2 mL ampules or vials)
 - b. Ketamine (Ketalar) — 2500 mg (5x 500 mcg/10 mL ampules or vials) or (5x 500 mcg/5 mL ampuls or vials)
 - c. Midazolam (Versed) — 60 mg (12x 5mg/1mL ampules or vials)
 - d. Morphine Sulfate (alternative) — N/A

Accountability for Expired or Damaged Controlled Substances

- 1. Note the expiration date or damage, the date of replacement, and the source of restock on the department's-controlled substance disposition and restock record (CMS EMS-I).
- 2. Place the expired drug or damaged container in the supplied container and label with the type of drug, dosage, unit identifier, and name of the paramedic who returned the drug and date.
- 3. Put the container in the Master Supply Storage to be sent back via reverse distribution to the designated supplier.
- 4. EMS Coordinator must fill out DEA 222 to accompany any reverse distribution.
- 5. Record the exchange of damaged or expired controlled substances using the forms CMS EMS-I and CMS EMS-2.
- 6. Only the Medical Director and/or EMS coordinator may remove outdated controlled substances or dangerous devices from the controlled substance Master Supply Storage located at fire station 1. Any items removed shall be sent back to a licensed designated supplier for disposal (Business and Professions Code 4119.01).

Accountability for Theft or Loss

- 1. Immediate notification to Captain, Duty Chief, EMS Coordinator and Chief.
- 2. Dual reverification of all the controlled substances will be conducted by two parties other than the original firefighter/paramedics.
 - a. Original firefighter/paramedics will remain present during reverification.
 - b. Inspection of all the CMS EMS-I and CMS EMS-2 documentation.
- 3. EMS Coordinator will follow the procedure outlined in accordance with REMSA and DEA form 106 (Theft/Loss Report of Controlled Substance)
 - a. Report must be submitted to our Local Diversion Field Office within 24 hours

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Reverse Distribution

1. Each agency or service will send expired and / or damaged controlled substances to an authorized reverse distributor. Schedule II controlled substances must be transferred using the DEA's Form 222 or the Controlled Substance Ordering System (CSOS), while Schedule III — V controlled substances may be transferred by invoice. These reverse distributions will be sent to the reverse distributor's facility found at the single physical location and address noted on the reverse distributor's DEA license. Each reverse distribution must be tracked in a manner that documents the parties sending and receiving the expired and / or damaged controlled substances.
2. Disposal of expired and / or damaged controlled substances "wasted" container will be performed as described above under "Reverse Distribution".

Documentation Accountability for Controlled Substances

1. ALL Controlled Substance documentation will remain in the Master Supply Storage.
 - a. DEA 223 — Medical Director and City of Calimesa Fire Department number
 - b. DEA 222 — Controlled Substance order form
 - c. CMS EMS-I — Daily Controlled Substance Inventory Log
 - d. CMS EMS-2 — Controlled Substance Restock Request Form
 - e. CMS EMS-3 - Monthly Expiration Controlled Substance Log
2. ALL Controlled Substance forms will remain in the Master Supply Storage for the calendar year.
 - a. After one year the controlled substance forms will be pulled, scanned, and stored for three (3) years.
3. ALL controlled substance forms will be dual inspected at shift change by ingoing and off going firefighter/paramedics.
 - a. CMSE1/CMSE2/CMSPTL1/Master Supply Storage
4. ALL controlled substance forms will be inspected by on-duty fire captain for completeness and compliance.
 - a. CMSE1/CMSE2/CMSPTL1/Master Supply Storage

Usage of Controlled Substance Procedures and Documentation

1. A log(s) of all controlled substances administered
2. A PCR/ePCR corresponding to each administration
3. These records will be:

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- a. Maintained with the medications until submitted to and / or electronically accessible from the master supply location
 - b. Available for inspection within twenty-four (24) hours
 - c. Submitted to master supply at least once (Ix) per month
 - d. Maintained as master supply documentation for a period of no less than three (3) years
4. Disposal of Controlled Substances:
- a. Disposal of controlled substances residual to patient administration ("wasting") will be performed following the agency or service's internal policy.
 - b. Personnel wasting controlled substances must be authorized by the DEA registrant and included on the agency or service's roster of personnel authorized to administer controlled substances. A witness, preferably included on that same roster, but, at minimum, included on the roster of personnel authorized to count controlled substances, must also participate in each waste and its documentation.
5. Disposal/Wasting documentation will include:
- a. A log(s) of all controlled substances wasted
 - b. A PCR/ePCR corresponding to each waste
 - c. These records will be:
 - i. Maintained with the medications until submitted to and / or electronically accessible from the master supply location
 - ii. Available for inspection within twenty-four (24) hours
 - iii. Submitted to master supply at least once (Ix) per month
 - iv. Maintained as master supply documentation for a period of no less than three (3) years
6. Usage/Disposal/Wasting Audits
- a. Be conducted by the DEA registrant or designee (EMS Coordinator or Medical Director)
 - b. Account for the current disposition of all controlled substances
 - c. Include review of forms, purchase records, logs, and PCRs/ePCR's
 - i. Identify and report discrepancies as required
 - d. Identify and investigate unusually high rates of administration
 - e. Be performed at least quarterly. Records of these audits will be:
 - i. Maintained at and / or electronically accessible from the agency or service's quality assurance location
 - ii. Available for inspection within twenty-four (24) hours
 - iv. Retained for a period of no less than three (3) years
7. Missing/Incorrect Number/Accidental Breaking of Accountability Tag
- If a narcotic accountability tag is found to be missing, damaged or has a non-matching number on CMS EMS-1 or CMS EMS-2 immediately notify the on-duty battalion chief and contact the EMS coordinator. An inspection of the case,

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narcotics and narcotic documentation will be conducted until the error is located and all narcotics are accounted for.

If a narcotic accountability tag is accidentally broken immediately notify the on-duty battalion chief and notify the EMS coordinator. An inspection of the case and narcotics will be conducted and if all narcotics are accounted for and the case is in-service a new tag will be placed, and the incident will be documented in the notation section at the bottom of the CMS EMS-1 form.

8. Discrepancy Reporting

Each agency or service will follow its internal policy for reporting discrepancies including tampering, theft, loss, or diversion of controlled substances. This policy will be established by the DEA registrant and must include immediate verbal reporting followed by written reports and investigation. The DEA registrant must notify the DEA of the discrepancy within one (1) business day of discovery, using either the paper form #106, "Report of Theft or Loss of Controlled Substances,"

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References

1. Riverside County Emergency Medical Services Agency Policy — 3302
2. Drug Enforcement Agency — Substance Controlled Schedules
3. Drug Enforcement Agency Form 106 Report of Theft/Loss of Controlled Substance

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Purpose

The purpose of this guideline is to ensure that all City of Calimesa Fire Department personnel who have access to patient information, understand the organization's concern for the respect of patient privacy and are trained and annually updated in accordance with OSHA guidelines regarding HIPPA policy and procedures.

Policy

1. All new City of Calimesa Fire Department personnel as part of their orientation will attend the mandatory two hour Health Insurance Portability and Accountability (HIPPA) training.
2. All current City of Calimesa Fire Department personnel will attend an annual HIPPA update class that will incorporate an instructor based power point and personnel participating scenarios.
3. All initial and update classes will be conducted by either the on-duty battalion chief or EMS coordinator.

Procedure

1. All attendees will receive copies of CMS's policies and procedures regarding privacy.
2. All attendees will receive copies of City of Calimesa Fire Department (CMS) policies and procedures regarding patient privacy.
3. All attendees must attend the training in person and verify attendance and agreement adherence to CMS policies and procedures on patient privacy.
4. Topics of the training will include a complete review of CMS's policy on privacy practices and will include other information concerning the HIPPA Act of 1996.
 - a. Overview of the federal and state laws concerning patient privacy including the privacy regulations under the HIPPA Act of 1996.
 - b. Description of protected health information (PHI).
 - c. Patients' rights under the HIPPA Act.
 - d. Personnel responsibilities under the HIPPA Act.
 - e. Importance of and benefits of privacy compliance.
 - f. Consequences of failure to comply with City of Calimesa's established policies and procedures.

CMS retains strict requirements on the security, access, disclosure and use of PHI. Access, disclosure and use of PHI will be based on the role of the individual personnel in the organization and should be only to the extent that the person needs access to PHI to complete necessary job functions. When PHI is accessed, disclosed and used, the individuals involved will make every effort, except in patient care situations, to only access, disclose and use PHI to the extent that only the minimum necessary information is used to accomplish the intended purpose. Patients may exercise their rights to access, amend, restrict, and request an accounting, as well as lodge a complaint with either CMS or Riverside Emergency Medical Services Agency. CMS

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maintains strict requirements on the security and access of all Electronic Patient Care Records (ePCR) as well as the initial documentation created by the field providers in their preparation of an ePCR.

1. All preliminary documentation used by CMS personnel to assist in the creation or modification of a PCR is the sole property of CMS.
2. Each crewmember will be given a password to use CMS's records management.
3. Each CMS personnel will have their own personal password to use on CMS records management system (RMS) or ImageTrend.
4. No CMS personnel may disclose his/her password to any other CMS personnel.
5. No CMS personnel is to log onto any computer or password protected software under any user other than his/her own.
6. An ePCR may be amended by a the ePCR originator with the approval of the EMS coordinator.
7. Any record contained in the Riverside EMS information system (REMSIS) version of Elite may be made available to system participants (prehospital/hospital) for shared patients through established practices when that system participant has a clinical need related to direct patient care or if/when required by the Riverside County EMS System Quality Improvement program and associated policies.

Incidental Disclosures

The City of Calimesa Fire Department understands that there will be times when there are incidental disclosures about PHI in the context of caring for a patient. The privacy laws were not intended to impede common health care practices that are essential in providing health care to the individual. Incidental disclosures are inevitable, but these will typically occur in radio or face-to-face conversation between health care providers, or when patient care information in written or electronic form and is left out in the open for others to access or see. The fundamental principle is that all personnel need to be sensitive about the importance of maintaining the confidence and security of all material we create or use that contains patient care information. CMS personnel should not have access to information that is not necessary for the personnel to complete his or her job. For example, it is generally not appropriate for field personnel to have access to billing records of the patient. However, all personnel must be sensitive to avoiding incidental disclosures to other health care providers and others who do not have a need to know the information. Pay attention to who is within earshot when you make verbal statements about a patient's health information, and follow some of these commonsense procedures for avoiding accidental or inadvertent disclosures:

City of Calimesa personnel should only discuss patient care information with those who are involved in the care of the patient, regardless of your physical location. You should be sensitive to your level of voice and to the fact that others may be in the area when you are speaking. This approach is not meant to impede anyone's ability to speak with other health care providers freely when engaged in the care of the patient. When it comes to treatment of the patient, you should be

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free to discuss all aspects of the patient's medical condition, treatment provided, and any of their health information you may have in your possession with others involved in the care of the patient.

Penalties for Violation

The City of Calimesa Fire Department takes its responsibility to safeguard patient information very seriously. There are significant legal penalties against agencies and individuals that do not adhere to the laws that protect patient information privacy.

Personnel who do not follow the policies and procedures of patient privacy will be subject to disciplinary action, up to and including counselling, verbal reprimands, written reprimands, suspension, demotion and/or termination in accordance with the department's SOP/SOG.

The City of Calimesa shall make every effort to provide remedial education and training as to our policies and procedures when there is a first time violation of policy.

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Policy: 502.02 Bloodborne Pathogen Compliance

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Purpose

The purpose of this guideline is to eliminate or minimize exposure to potentially infectious blood borne pathogens in accordance with the CFR29, Part 1910.1030, and become familiar with Body Substance Isolation (BSI) procedures.

Policy

All personnel with the potential for occupational exposure will participate in a mandatory training program related to exposure.

1. All new City of Calimesa Fire Department personnel as part of their orientation will attend the mandatory two hour Bloodborne Pathogen (BBP) training.
2. All current City of Calimesa Fire Department personnel will attend an annual BBP update class that will incorporate an instructor based power point and personnel participating scenarios.
3. All initial and update classes will be conducted by either the on-duty battalion chief or EMS coordinator.
4. Reference SOP 501.05 Infectious Control for BBP identification and BSI/PPE.

Procedure

- All attendees will receive copies of CMS's policies and procedures regarding BBP and Incident Exposure Control.
- All attendees will receive copies of City of Calimesa Fire Department (CMS) policies and procedures regarding BBP and Incident Exposure Control.
- All attendees must attend the training in person and verify attendance and agreement adherence to CMS policies and procedures on patient privacy.
- Topics of the training will include a complete review of CMS's policy on recognition, prevention and reporting exposure.
 - Wearing one's PPE/BSI (gloves, masks & eye protection)
 - Wash hands after all incidents
 - HEPA or N95/N100 mask for potential high-risk respiratory patients
 - Decontaminating foreign substances or fluids on body or uniform
 - Sharps disposal
 - Decontaminating EMS gear after exposure
 - Use of red biohazardous bags for exposed gloves, masks, gowns, clothing, linen, backboards and straps, dressing and bandages.
 - All unknown secretions will be considered infectious.
- Exposure Reporting (Policy 501.06 Infectious and Exposure Control Plan)
 - Personnel will immediately report any exposure to his/her Captain.
 - Captain or Company Officer will immediately report the incident to the on-duty Battalion Chief and EMS Coordinator.

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- Fire Chief will be updated of the incident.
- Appropriate Workman's Compensation paperwork will be filled out and a City of Calimesa Fire Department exposure form.
- Personnel will be transported to the appropriate medical facility and seek evaluation.
- Personnel shall provide the names and contact information of all their emergency contacts.
- If the source patient has been transported to their appropriate medical facility, immediate contact of the receiving facility for advice on their procedures.
- If the receiving facility is allowed to follow their HIV exposure procedures both source patient and personnel will be tested within 90 days.
- The personnel will be provided counseling and evaluation of the reported exposure.
- Documents can be found in the City of Calimesa Fire Department's IIPP (Injury and Illness Prevention Program).